



Pediatric-Surgery-Exam-2105-...



12*. The curative treatment of the testicular torsion consists of:

- A. local administration of non-steroidal anti inflammatory drugs
- B. ice bag application in the first 6 hours
- C. emergency surgical intervention**
- D. antibiotic administration in high dosage
- E. sedation

51. Cephalhematoma:

- A. is a subperiosteal hemorrhagic collection;**
- B. is a subcutaneous hemorrhagic collection;
- C. is more frequent in the frontal area and has fluctuence;
- D. has a slow evolution toward resorption in 2-4 weeks with unaesthetic callus;**
- E. needs correction of hypo-prothrombinemia (vitamin K, C, Ca) and evacuation after few days**

52. Obstetrical cranial fractures:

- A. are often seen due to cartilaginous sutures that allows cranial deformity;
- B. are most frequent linear fractures;**
- C. are most frequent depressed fractures;
- D. are sometimes accompanied by cephalhaematoma;**
- E. depressed skull fractures beyond 5-6mm requires surgical correction because they can cause neurological disorders**

53. Vrolich Disease (osteogenesis imperfect- the congenital precocious form):

- A. is autosomal recessive transmitted;
- B. starts and evolves in the intrauterine life, the child presents with multiple fractures at birth;**
- C. is autosomal dominant transmitted;**
- D. it is known as the 'brittle bone disease'
- E. has a bad prognosis**

54. The structural classification of lymphangioma is:

- A. plain lymphangioma
- B. simple lymphangioma**
- C. cavernous lymphangioma**
- D. mesenteric lymphadenitis
- E. cystic lymphangioma or hygroma**



Pediatric-Surgery-Exam-2105-...



82. Acute scrotum includes the following:

- A. testicle torsion;**
- B. Morgagni testicular torsion;**
- C. retractile testicle;
- D. testicular and scrotal trauma;**
- E. acute scrotal edema**

83. Differential diagnosis for adnexial torsion in girls less than 5 years is made with:

- A. acute appendicitis;**
- B. renal colic;**
- C. orchiepididimitis;
- D. rupture of the ovarian cyst;**
- E. ectopic extra uterine pregnancy

84. Stage 1 varicocele:

- A. is evident through inspection;
- B. appears only in clinostatism;
- C. is evident only by palpation;**
- D. disappears only in clinostatism;
- E. do not implies macroscopic testicle modifications**

85. Pericardial tamponade:

- A. results from accumulation of air in the pericardial cavity;
- B. follows thoracic trauma with penetrated wound;**
- C. results from accumulation of blood in the pericardial cavity;**
- D. always has fatal evolution
- E. treatment is pericardiocentesis**

86. Cerebral concussion:

- A. results after minor cranio-cerebral trauma;
- B. lack of consciousness is transitory and for less than 10 min;**
- C. is determinate by perturbation of the cerebral functions due to an organic lesion;
- D. is spontaneous reversible;**
- E. it is possible to occur epilepsy.**

87. Hypertrophic pyloric stenosis is clinically manifested through:

- A. 2-3 weeks free interval from birth followed by bilious vomiting;
- B. 2-3 weeks free interval from birth followed by white**



Pediatric-Surgery-Exam-2105-...



70. Intestinal malrotation is present in:

- A. congenital inguinal hernia;
- B. omphalocele;**
- C. laparoschisis;**
- D. duodenal stenosis or intestinal atresia;**
- E. hypertrophic pyloric stenosis

71. Mesenteric lymphadenitis:

- A. is the most frequent cause of abdominal pain in infants;
- B. is represent by the inflammation of mesenteric ganglions from ileocolic region;**
- C. is represent by the inflammation of retroperitoneal ganglions;
- D. etiology is represented by viruses and bacteria**
- E. the involved ganglions are hypertrophied and associates fluid peritoneal reaction

72. Clinical picture in mesenteric lymphadenitis is based on:

- A. periumbilical pain in crisis that come over and over;**
- B. fever 39 degree Celsius with good general status;**
- C. with or without vomiting**
- D. polyuria and polydipsia
- E. localized pain in the ureteral points exactly identified by patient

73. Treatment of primitive peritonitis consists of:

- A. large dosis of Oxacilin (3-6 g/24h);
- B. biopsy of mesenteric ganglions;
- C. large dosis of Penicilin (3-6mil/24h)**
- D. if it is doubt surgery is indicated;**
- E. if neglected with cystic peritonitis drainage and antibiotics are associated.**

74. Simple meconium ileus:

- A. is localized in the proximal portion of the small bowel;
- B. is localized in the distal portion of the small bowel;**
- C. the proximal segment is dilated (7-10cm) with muscular hypertrophy + the meconium is viscous and adherent;**
- D. distal ileon has a reduced calibre with storage of meconium like pearls;**
- E. large bowel has a microcolon aspect contains mucus and meconium**



Pediatric-Surgery-Exam-2105-...



65. In the umbilical urachal fistula:

- A. there is a communication between umbilicus and gallbladder;
- B. one can notice clear fluid (urine) at the umbilical button;**
- C. one can notice intestinal fluid at the umbilical button
- D. urine examination demonstrate urinary infection of the bladder**
- E. surgical treatment consist of double ligation and resection of the fistula**

66. The duodenal malformations could associate one or more of the following:

- A. mongolism (Down Syndrome) in 30% of cases;**
- B. cardiac malformations;**
- C. venous malformations
- D. small intestine malformations**
- E. prematurity**

67. Intussusception:

- A. represent introduction of a portion of small intestine through a mesenteric defect;
- B. represent the protrusion of a suprajacent segment into subjacent segment of small intestine**
- C. result in mechanical intestinal obstruction;**
- D. it is more frequent in newborns;
- E. it is more frequent in infants.**

68. Hirschsprung disease present with:

- A. the absence of intestinal transit;**
- B. progressive abdominal distension;**
- C. bulimia;
- D. vomiting-initial food, then bilious and at the end feces;**
- E. bad smelling soft stools and paradoxical diarrhea**

69. In anal stenosis:

- A. anal orifice is normally located;**
- B. anal orifice is ectopic;
- C. anal channel is incompletely block by a mucus diaphragm;**
- D. anal channel is completely blocked by a mucus diaphragm;
- E. the feces has a thin pen like aspect.**

70. Intestinal malrotation is present in:



Pediatric-Surgery-Exam-2105-...



1*. The complications of hemangiomas are the following with exception:

- A. exulceration
- B. infection
- C. malignization**
- D. hemorrhage
- E. thrombocytopenia (Kassabach-Meritt)

2*. In type I esophageal atresia:

- A. both esophageal segments have a communication with trachea;
- B. the abdomen is progressively distending due to air accumulation in the stomach;
- C. the esophageal segments have no link to trachea;**
- D. the introduction of a nasogastric tube stops nausea;
- E. the feeding of newborn will be done in little amounts every 2 hours

3*. The preferred surgical technique in the hiatal hernia is:

- A. gastropexia
- B. fundic plication of Nissen;**
- C. subtotal esofagoplasty
- D. subtotal gastroectomy
- E. no surgical treatment is necessary, only medical

4*. The typical contrast imaging in duodenal atresia shows:

- A. narrowing and elongation of the pyloric segment
- B. 'corkscrew' image
- C. 'double bubble' in the superior abdomen**
- D. A+B
- E. A+C

5*. In type I interstitial atresia, one may notice:

- A. the continuity of the sero-muscular wall with a membrane blocking the lumen**
- B. between the closed segments of the gut there is a fibrous cord, the mesenter is intact or has a 'V' interruption;
- C. the closed interstitial segments are completely separated, the mesenteric defect is in 'V'
- D. multiple completely separated atretic areas, the 'V' shaped mesenteric defects take the aspect of a 'pagoda'
- E. none of the above mentioned aspects

6*. The tumour in intussusception:



Pediatric-Surgery-Exam-2105-...



6*. The tumour in intussusception:

- A. is composed by 2 cylinders
- B. is composed by 5 cylinders
- C. is composed by 4 cylinders
- D. has a intussuscepted cylinder, one is recipient and one intermediary;**
- E. the external cylinder is formed on behalf of the penetrating or receiving loop

7*. The most frequent affected region in the congenital megacolon is:

- A. cecal
- B. ileal
- C. recto-sigmoidian**
- D. jejunal
- E. anal

8*. The X-ray with barium in intestinal malrotation with volvulus shows:

- A. 'double bubble' image
- B. 'corkscrew' image**
- C. opacification of the whole colic frame
- D. the retention of the contrast in the volvulated intestine
- E. the presence of the dye above and below the volvulated intestine

9*. The primitive peritonitis are most frequent

- A. staphylococcus
- B. pneumococcus**
- C. gonococcus
- D. meningococcus
- E. streptococcus

10*. The most frequent localization of the hydatid cyst is:

- A. in the lungs
- B. in the liver**
- C. in bones
- D. in the spleen
- E. in muscles

11*. The sacrococcygeal teratoma has his origin in:

- A. ectoderma
- B. endoderma
- C. mesoderma
- D. ectoderma, endoderma and mesoderma**



Pediatric-Surgery-Exam-2105-...



60. The differential diagnosis of hypertrophic pyloric stenosis consist of:

- A. feeding mistakes;**
- B. regurgitation**
- C. milk intolerance**
- D. ano-rectal malformation;
- E. meconium ileus.

61. The major form of omphalocele consists of:

- A. Parietal defect less than 2cm;
- B. parietal defect less than 5cm;
- C. hernia of the intestine and a part of liver;**
- D. the dimensions of omphalocele causes difficult expulsion of the fetus and membrane rupture risk;**
- E. edema and retraction of the intestinal loop

62. Laparoschisis presents with:

- A. right paraumbilical parietal defect 2-3 cm, through which herniates the small and large intestine;**
- B. absent umbilical cord;
- C. malrotation of small intestine through lack of adhesion, intestinal loops are edematous, enlarged and grey due to the contact with amniotic fluid;**
- D. shorter intestine (up to 1/3 of normal length)**
- E. very rare associated with prematurity.

63. Omphaloenteric fistula:

- A. represent the persistence of internal portion of omphaloenteric duct**
- B. is covered with secretor digestive mucosa that produce a serous secretion**
- C. exploration is made with a metallic stilet or radiologic examination with contrast in profile exposure;
- D. the treatment is surgical and consist of fistula excision**
- E. surgery is not necessary

64. Meckel's diverticulum:

- A. is a rest of the omphalomesenteric duct present in 1.5% of population;**
- B. usually asymptomatic is diagnosed by the associated complications;**
- C. present gastric mucosal islets (duodenal, jejunal, colic or pancreatic mucosa) and in the interior has intestinal mucosa;**
- D. has a dimension between 1-8m;



Pediatric-Surgery-Exam-2105-...



- A. is evident through inspection;
- B. appears only in clinostatism;
- C. is evident only by palpation;**
- D. disappears only in clinostatism;
- E. do not implies macroscopic testicle modifications**

85. Pericardial tamponade:

- A. results from accumulation of air in the pericardial cavity;
- B. follows thoracic trauma with penetrated wound;**
- C. results from accumulation of blood in the pericardial cavity;**
- D. always has fatal evolution
- E. treatment is pericardiocentesis**

86. Cerebral concussion:

- A. results after minor cranio-cerebral trauma;
- B. lack of consciousness is transitory and for less than 10 min;**
- C. is determinate by perturbation of the cerebral functions due to an organic lesion;
- D. is spontaneous reversible;**
- E. it is possible to occur epilepsy.**

87. Hypertrophic pyloric stenosis is clinically manifested through:

- A. 2-3 weeks free interval from birth followed by bilious vomiting;
- B. 2-3 weeks free interval from birth followed by white projectile vomiting;**
- C. as the stenosis progress the vomiting becomes bilious;
- D. immediately after vomiting the child wants to eat again;**
- E. as the disease progress the vomiting becomes less frequent but more abundant**

88. The testicular tumors treatment includes:

- A. surgical treatment;**
- B. chemotherapy**
- C. cobaltotherapy**
- D. immunotherapy**
- E. antibiotherapy



Pediatric-Surgery-Exam-2105-...



- A. is evident through inspection;
- B. appears only in clinostatism;
- C. is evident only by palpation;**
- D. disappears only in clinostatism;
- E. do not implies macroscopic testicle modifications**

85. Pericardial tamponade:

- A. results from accumulation of air in the pericardial cavity;
- B. follows thoracic trauma with penetrated wound;**
- C. results from accumulation of blood in the pericardial cavity;**
- D. always has fatal evolution
- E. treatment is pericardiocentesis**

86. Cerebral concussion:

- A. results after minor cranio-cerebral trauma;
- B. lack of consciousness is transitory and for less than 10 min;**
- C. is determinate by perturbation of the cerebral functions due to an organic lesion;
- D. is spontaneous reversible;**
- E. it is possible to occur epilepsy.**

87. Hypertrophic pyloric stenosis is clinically manifested through:

- A. 2-3 weeks free interval from birth followed by bilious vomiting;
- B. 2-3 weeks free interval from birth followed by white projectile vomiting;**
- C. as the stenosis progress the vomiting becomes bilious;
- D. immediately after vomiting the child wants to eat again;**
- E. as the disease progress the vomiting becomes less frequent but more abundant**

88. The testicular tumors treatment includes:

- A. surgical treatment;**
- B. chemotherapy**
- C. cobaltotherapy**
- D. immunotherapy**
- E. antibiotherapy



Pediatric-Surgery-Exam-2105-...



82. Acute scrotum includes the following:

- A. testicle torsion;**
- B. Morgagni testicular torsion;**
- C. retractile testicle;
- D. testicular and scrotal trauma;**
- E. acute scrotal edema**

83. Differential diagnosis for adnexial torsion in girls less than 5 years is made with:

- A. acute appendicitis;**
- B. renal colic;**
- C. orchiepididimitis;
- D. rupture of the ovarian cyst;**
- E. ectopic extra uterine pregnancy

84. Stage 1 varicocele:

- A. is evident through inspection;
- B. appears only in clinostatism;
- C. is evident only by palpation;**
- D. disappears only in clinostatism;
- E. do not implies macroscopic testicle modifications**

85. Pericardial tamponade:

- A. results from accumulation of air in the pericardial cavity;
- B. follows thoracic trauma with penetrated wound;**
- C. results from accumulation of blood in the pericardial cavity;**
- D. always has fatal evolution
- E. treatment is pericardiocentesis**

86. Cerebral concussion:

- A. results after minor cranio-cerebral trauma;
- B. lack of consciousness is transitory and for less than 10 min;**
- C. is determinate by perturbation of the cerebral functions due to an organic lesion;
- D. is spontaneous reversible;**
- E. it is possible to occur epilepsy.**

87. Hypertrophic pyloric stenosis is clinically manifested through:

- A. 2-3 weeks free interval from birth followed by bilious vomiting;
- B. 2-3 weeks free interval from birth followed by white**



Pediatric-Surgery-Exam-2105-...

**E. cystic lymphangioma or hygroma**

55. The cyst and fistulas derives from the second brachial arch;

A. are more frequent than those derive from the first brachial arch;

B. may be uni- or bilateral;

C. are located on the anterior margin of sternocleidomastoidian muscle;

D. are located on the anterior margin of deltoid muscle

E. can be tolerated long period, but get infected by time

56. Meningoencephaloceles:

A. is more frequent than myelomeningoceles;

B. is located mostly lumbar;

C. is produced on the median, frontonasal and occipital line;

D. occurs due to the suture defect of the cranial bones;

E. one or more vertebrae have the medullar channel open.

57. Esophageal atresia:

A. has absent esophageal lumen in his medial third;

B. is the most frequent malformation of the urinary system;

C. is incompatible with life;

D. sex ration is equal;

E. the lesion occurs at the embryo between 24-27 weeks

58. Congenital diaphragmatic left hernia presents with:

A. enlarged peritoneal cavity;

B. depressed, hypoplastic and pushed left lung toward the pleural dome;

C. the lung herniates in the abdomen;

D. the heart and mediastinum are deviated to the right;

E. the abdominal viscera protrude in pleural cavity

59. Hiatal hernia:

A. appears through the Morgagni foramen;

B. appears though the Bochdalek foramen;

C. appears through esophageal hiatus;

D. associated with hypertrophic pyloric forms Roviralta syndrome;

E. appears through foramen ovale.

60. The differential diagnosis of hypertrophic pyloric stenosis consist of:



Pediatric-Surgery-Exam-2105-...



75. Differential diagnosis for meconium ileus is made with:

- A. duodenal atresia;
- B. megacolon;**
- C. mesenteric lymphadenitis;
- D. left small colon;**
- E. colic atresia**

76. Inguinal hernia:

- A. the most frequent cases are diagnosed at premature;**
- B. is more frequent in girls;
- C. is never bilateral;
- D. is more frequent bilateral;
- E. is more frequent in boys**

77. Inguinal hernia in the school child:

- A. is visible only at the effort and coughing (excepting the large incoercible hernias);**
- B. results from failure to obliterate of the umbilical ring;
- C. is always bilateral;
- D. at physical examination one can observe a large external orifice of the inguinal channel at the affected side;**
- E. for all male cases is mandatory to check the presence of the testicles in the scrotum**

78. Clinical picture in pulmonary hydatid cyst consist of:

- A. coughing-initial is irritative and than is mucopurulent;**
- B. dyspnoea-always;
- C. hemoptisia-repetative;**
- D. allergic skin modifications (urticaria) to some patients;**
- E. intercostal pain if a pleural reaction is present**

79. Nephroblastoma is clinically manifested as:

- A. variable dimension tumor with lumbar contact, firm, easy to mobilize;**
- B. vague pain, sometimes irradiating on the thigh, with limping;**
- C. sometimes macroscopic hematuria;**
- D. Blood hypertension due to renal artery compression;**
- E. diarrhea

80. Ureteral anomalies in calibre and shape include:

- A. retrocaval ureter;
- B. ureteral diverticulum;**
- C. ureterocel;
- D. congenial ureteral stenosis**
- E. congenital megaureter**



Conectați-vă pentru a edita acest fișier și a salva ...



88. Tratamentul tumorilor testiculare include:

A. Tratament chirurgical

B. Chimioterapie

C. Terapie cu cobalt

D. Imunoterapie

E. antibioterapie

grile-engl-2015-traduse - Doar în citire



Conectați-vă pentru a edita acest fișier și a salva ...



C. Varsaturile pot fi frecvente sau pot lipsi

D. Poliurie și polidipsie

E. Durerea apare în punctele ureterale și este indentificată exact de către pacient

73. Tratamentul în peritonita primitivă:

A. Doze mari de Oxacilina (3-6 mil. U.I/zi)

B. Biopsia ganglionilor mezenterici

C. Doze mari de penicilina (3-6 mil. U.I/zi)

D. În caz de dubiu de intervenție chirurgicală

E. În cazurile neglijate cu peritonită închistată se recomandă antibioterapie și drenarea colecției

74. Ileusul meconial simplu:

A. Se localizează în porțiunea proximală a intestinului subțire

B. Este localizată în porțiunea distală a intestinului subțire

C. Segmentul proximal este dilatat (7-10 cm) cu hipertrofie musculară + meconiul este vascos și aderent

D. Ileonul distal este de calibru redus cu depozite de meconiu sub formă de margele

E. Colonul are aspect de microcolon cu depozite de mucus și meconiu

75. Diagnosticul diferențial al ileusului meconial se face cu:

A. Atrezia duodenală

B. Megacolonul

C. Limfadenită mezenterică

D. Sindrom de colon mic stâng iritabil

E. Atrezia colică

76. Hernia inghinală:



Conectați-vă pentru a edita acest fișier și a salva ...



76. Hernia inghinală

A. Cele mai multe cazuri sunt diagnosticate la prematur

B. Mai frecvent la fete

C. Nu este niciodată bilaterală

D. Este mai frecventă bilateral

E. Apare mai frecvent la băieți

77. Hernia inghinală la sexul feminin

A. Este vizibilă la efort și tuse (excepție face hernia strangulată)

B. Rezultă din imposibilitatea de închidere a inelului ombilical

C. Este întotdeauna bilaterală

D. La examenul obiectiv se poate observa un orificiu extern al canalului inghinal lărgit pe partea afectată

E. Pentru toate cazurile care apar la sexul masculin este obligatorie verificarea scrotului pentru a depista testiculele

78. Tabloul clinic în chistul hidatic pulmonar:

A. Tuse- inițial iritativă apoi devine mucopurulentă

B. Dispnee-întotdeauna

C. Hemoptizia-repetitivă

D. Alergii cutanate (urticarie) la unii pacienți

E. Durere intercostală dacă apare o pleurezie

79. Nefroblastomul se manifestă clinic prin:

A. Tumora de dimensiuni variabile cu contact lombar, fermă, ușor de mobilizat

B. Durere vagă, cu caracter iritant, cu iradiere în coapsă, cu mers schiopătat

C. Uneori hematurie macroscopică

D. Hipertensiune arterială datorită compresiei arterei renale

E. Diaree

80. Anomaliile ureterale de calibru și formă includ:

A. Ureterul retrocav



Conectați-vă pentru a edita acest fișier și a salva ...



Anomaliile ureterale de calibru și forma includ:

A. Ureterul retrocav

B. Diverticulul ureteral

C. Ureterocelul

D. Stenoza ureterala congenitala

E. Megaureterul congenital

81. Refluzul vezico-ureteral:

A. Reprezinta intoarcerea urinei din vezica urinara in tractul urinar superior

B. Este intoarcerea urinei din ureter in vezica urinara

C. Este secundara bolii Hirshprung

D. Este diagnosticata prin cistografie mictionala

E. Necesita reimplantarea antireflux vezicoureterala

82. Scrotul acut include:

A. Torsiunea de testicul

B. Torsiunea de testicul Morgagni

C. Retractia testiculara

D. Trauma testiculara si scrotala

E. Edemul scrotal acut

83. Diagnosticul diferential al torsiunii anexiale la fetite sub 5 ani se face cu:

A. Apendicita acuta

B. Colica renala

C. Orhiepididimita

D. Ruptura unui chist ovarian

E. Sarcina ectopica

84. Varicocelul de gradul I:

A. Este evident prin inspectie

B. Apare doar in clinostatism

C. Este evident doar prin palpare

grile-engl-2015-traduse - Doar în citire



Conectați-vă pentru a edita acest fișier și a salva ...



B. Apare doar în clinostatism

C. Este evident doar prin palpare

D. Dispare doar în clinostatism

E. Nu implică modificări testiculare macroscopice

85. Tamponada pericardică:

A. Rezultă în urma acumulării de aer în cavitatea pericardică

B. Urmează unei traume toracice cu leziune penetrantă

C. Rezultă în urma acumulării de sânge în cavitatea pericardică

D. Are întotdeauna evoluție fatală

E. Tratamentul este pericardiocenteza

86. Comotia cerebrală:

A. Rezultă în urma unui traumatism minor

B. Alterarea stării de conștiință este tranzitorie cu o durată sub 10 minute

C. Este determinată de perturbarea funcțiilor cerebrale datorită unei leziuni organice

D. Este reversibilă spontan

E. La distanță poate să apară epilepsia

87. Tabloul clinic în stenoza hipertrofică de pilor:

A. 2-3 săptămâni de interval liber între nașterea și începutul varsăturilor biliase

B. 2-3 săptămâni de interval liber între naștere și varsături albe în jet

C. Pe măsura ce stenoza progresează, varsăturile devin biliase

D. Imediat după ce varsă copilul dorește realimentarea

E. Pe măsura ce boala evoluează, varsăturile devin mai rare, dar mai abundente

88. Tratamentul tumorilor testiculare include:

A. Tratament chirurgical

B. Chimioterapie

C. Terapie cu cobalt

D. Radioterapie